10.1 Iron-Deficiency Anemia*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I get tired and breathless climbing stairs." **Objective** • Hb 8.5 g/dL; low ferritin • Pale conjunctivae • HR 104/min on exertion	**ACTUAL** Decreased activity tolerance related to reduced oxygen-carrying capacity as evidenced by exertional dyspnea, fatigue and low hemoglobin.	• Manages ADL without breathlessness within 2 weeks of treatment.	• Plan activities with rest in between. • Check HR and SpO_2 during activity. • Give iron as prescribed; watch for reactions if given IV. • Follow hemoglobin and ferritin.	• Met when ADL are tolerated.
Subjective • "I do not eat meat often." **Objective** • Little iron in the diet • Heavy periods	**ACTUAL** Inadequate health knowledge related to dietary iron and oral iron therapy as evidenced by a low-iron diet and questioning.	• Names iron-rich foods and explains how to take oral iron.	• Teach which foods are rich in iron: red meat, liver, legumes, dark green leaves, dates. • Advise taking iron with vitamin C, not with tea, milk or antacids. • Explain that dark stools and constipation are to be expected. • Refer for investigation of the blood loss.	• Achieved when she repeats the advice.
Subjective *No specific complaint* **Objective** • Taking oral iron • Low-fiber diet	**RISK** Risk for impaired intestinal elimination as evidenced by risk factors: oral iron therapy.	• Soft stools at least every 2–3 days during treatment.	• Encourage fluid and fiber. • Note her bowel pattern. • Give a stool softener if ordered.	• Outcome reached when the bowel pattern is regular.

* **References for this care plan:** [32], [35], [37], [108]. Full entries are listed in the References section of the CPMN book.